NURSING DEPARTMENT

Policy Name : Assessment of Newborn

Policy Number : AON- NICU 3/3 (8)

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#	Type of Edition	Date of Revision	Effective Date	Due Review Date
8	8th Edition	10/09/2024	10/11/2024	11/11/2027
7	7th Edition	10/10/2021	10/10/2021	11/10/2024

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1. **Purpose**
    a. To determine any apparent problems and to identify those that demand immediate attention.
    b. To evaluate cardiopulmonary and neurologic function.
    c. To evaluate any obvious congenital anomalies or evidence of neonatal distress.

2. **Scope**
    a. Routine for all neonates admitted in NICU.
    b. Documenting in the newborn maturity rating and classification form.
3. **Definition**
    a. Assessment of Newborn:
    It is an identification by a nurse of the needs, preferences and abilities of a client. Assessment includes an interview with and observation of a client by the nurse and considers the symptoms and signs of the condition, the client verbal and nonverbal communication, the client's medical and social history, and any other information available. Assessment is extremely important because it provides the scientific basis for a complete nursing care plan.
4. **Abbreviation**:
    a. NICU - Neonatal lntensive Care Unit
5. **Policy Statements**
    a. Upon admission, newborn will be assessed for physical health, determination of any abnormalities that may exist, while establishing and maintaining the health and well-being of the newborn.
    b. A thorough and systemic physical assessment is an essential component in the care of a high risk infant.
    c. The nurse should be aware and alert of the subtle changes and reacts promptly to implement interventions that promote optimum functioning in the high risk neonate.
    d. Accurate documentation of the infant's status should be performed.

6. **Procedure:**
    a. Material/Equipment
        i. Radiant Warmer
        ii. Gloves
        iii. Stethoscope
        iv. Thermometer
        v. Oxygen and suction equipment

vi. Measuring tape
vii. Cardiac monitor
viii. Newborn assessment form
ix. Ophthalmic antibiotic ointment
x. Footprint ink set
xi. Radiant warmer, isolette or other controlled-heating device

Procedure	Rationale
1. Complete physical and gestational age assessment as soon as possible after delivery.	Completing the assessment serve as a guide in the plan of care.
2. Initial assessment should include, but is not limited to; 2.1. Estimation of gestational age by evaluation of both neuromuscular and physical maturity. 2.2. Determination of presence of anomalies of previously unsuspected disease, as well as general status of the infant.	Assessing the gestational age is important in providing total care to the patients. Newborns are examined after birth to check for major abnormalities and to make sure extra uterine life is without difficulty.
3. Assess the infant of the following: a. Skin and subcutaneous tissue b. Head, Neck and mouth c. Chest and abdomen d. Genitalia and anus e. Extremities, spine and joints f. Respiratory status, rate g. Breath sound h. Cardiovascular system	To obtain baseline information of the patient's vital signs, presence of deformity on the mentioned body areas.
4. Evaluate neurological status; a. Cry b. Muscle tone c. Symmetry of movement d. Plantar grasp e. Motor reflex f. Sucking and rooting reflexes	Assessing the neurological status of the patient is necessary in planning the care of patient and the need for immediate and further intervention.
5. Assess the following: a. Weight documented in grams and pounds. b. Length documented in centimeters and c. inches. d. Head circumferences in centimeters and e. inches.	Obtaining these initial parameters serve as a baseline data and guide in the plan of care, calculating the patient's drug dose and the prompt treatment required.

f. Abdominal girth in centimeters and inches. g. Vital signs as well as blood pressures, if h. appropriate. i. Oxygen saturation. j. ECG strip k. IV lines in place	
6. Assess for pain management.	
7. Assess for any vaccination given.	
8. Document all findings and report any unusual findings to the physician	Any unusual findings should be reported immediately for prompt treatment.
REASSESSMENT	
1. Re-assessment should commence within 30 minutes after the neonate receive any Surfactant or any high alert medications such as narcotics and controlled drugs are administered.	To determine the response to treatment.
2. Reassessment of ventilated patients are done 12 hours or more frequently especially those patients with respiratory care treatment has been modified (extubated to CPAP/ ventilator settings changes.	To determine the tolerance of new respiratory settings.
3. Any newborns must be re-assessed within 6 hours after initiating feeding.	To assess feeding intolerance.
4. Documented in the nurse's flow sheet.	

7. **Responsibilities**
    a. Attending Physician - Assess and document in the Newborn Maturity Rating Scale.
    b. Nurses - Assess and document in the flow sheet and admission assessment furm, refer any abnormal findings to the concerned physician.
8. **Documentation**
    a. Newborn Maturity Rating Scale and Classification
    b. Patient Physical Report (FM-004)
    c. Nurses Flow Chart (FM-051)
9. **Measurements**
    a. 100% of the newborns admitted in NICU are assessed and documented.
10. **References**
    a. http://medical-dictionary.thefreedictionary.com/nursingassessment
    b. Manual of Neonatology, 5th edition
11. **Cross References**
    a. CBAHI Standards
    b. Policies Procedures and Guidelines, Third Edition

12. **Attachments**
    N/A
13. **Distribution List**
    a. Nursing Department
    b. Quality Department
    c. Medical Administration
    d. Head of Pediatric Department
    e. Executive Director General
14. **Review Summary**
    a. Re-assessment